



Candida Vulvovaginitis in Adults

Treatment of Acute Infection

Vaginitis is an inflammation of the vagina that can result in discharge, itching, and pain. The cause is usually a change in the balance of vaginal bacteria or an infection. Reduced estrogen levels after menopause and some skin disorders can also cause vaginitis.

The most common types of vaginitis are:

- **Bacterial vaginosis:** This results from an overgrowth of the bacteria naturally found in the vagina, which upsets the natural balance.
- **Yeast infections:** These are usually caused by a naturally occurring fungus called *Candida albicans*.
- **Trichomoniasis:** This is caused by a parasite and is often sexually transmitted.

Treatment depends on the type of vaginitis you have.

Symptoms

Vaginitis signs and symptoms can include:

- Change in color, odor, or amount of discharge from your vagina
- Vaginal itching or irritation
- Pain during sex
- Painful urination
- Light vaginal bleeding or spotting

If you have vaginal discharge, the characteristics of the discharge might indicate the type of vaginitis you have. Examples include:

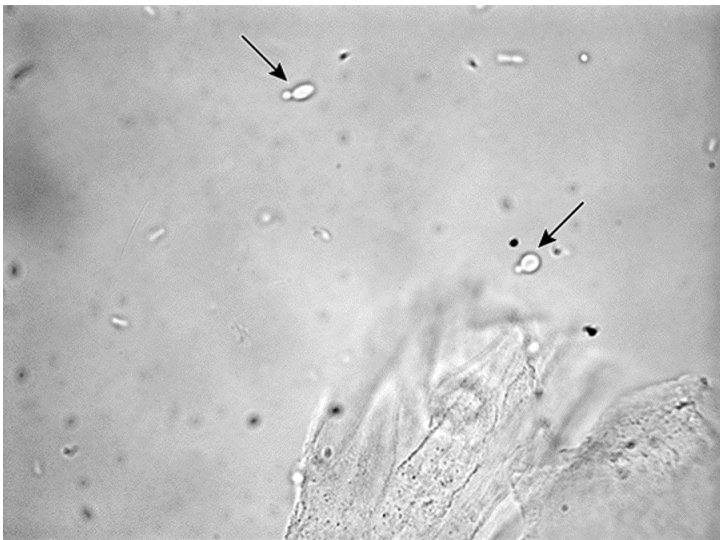
- **Bacterial vaginosis:** You might develop a grayish-white, foul-smelling discharge. The odor, often described as a fishy odor, might be more obvious after sex.
- **Yeast infection:** The main symptom is itching, but you might have a thick, white discharge that resembles cottage cheese.
- **Trichomoniasis:** This can cause a greenish-yellow, sometimes frothy, discharge.

Vulvovaginal candidiasis (VVC) is one of the most common causes of vulvovaginal itching and discharge. The disorder is characterized by inflammation in the setting of growth of *Candida* species. Treatment is indicated for the relief of symptoms and varies based on disease severity.

Diagnosis and Treatment

Determining the etiology of vaginitis is mandatory before initiating therapy. Please find out if symptoms are acute, chronic, or recurrent. Look for associated symptoms of pelvic pain and detail sexual history.

For **adult primary care**, we recommend doing a molecular vaginitis panel prior to starting empiric therapy.



Note. From *Budding yeast* [Photograph] by UpToDate, 2023, UpToDate (<https://www.uptodate.com/contents/image?imageKey=OBGYN%2F61326>).

immunocompromised, have infection with non-albicans *Candida* (NAC) species, or have recurrent infection (≥3 test-confirmed infections per year).

Consider the potential for STIs for patients who are sexually active while doing testing for vaginal discharge. Between 25-40% of patients presenting with vaginal symptoms will not have a specific cause identified on initial diagnostic evaluation. For continued or recurrent symptoms, please repeat testing.

Microscopic exam of normal vaginal discharge shows predominance of squamous epithelial cells and lactobacillus species. With *Candida* infection, it may show candida buds and hyphae.

Prescriptions include one or two doses of 150 mg Diflucan, and topical applications like Gyne-Lotrimin, Miconazole, or nystatin.

Patients with **complication infection** include those who are pregnant, have severe symptoms or are

Treatment of Complicated Vaginal Candidiasis	
Severe vaginitis symptoms	• Oral fluconazole 150 mg every 72 hours for two or three doses (depending on severity). • Topical azole antifungal therapy daily for 7 to 14 days. A low-potency topical corticosteroid can be applied to the vulva for 48 hours to relieve symptoms until the antifungal drug exerts its effect.
Non-albicans <i>Candida</i> vaginitis	Therapy depends upon species identified: • <i>C. glabrata</i>: Intravaginal boric acid* 600 mg daily for 14 days. • If failure occurs: 16% topical flucytosine cream, 5 g nightly for 14 days. • <i>C. krusei</i>: Intravaginal clotrimazole, miconazole, or terconazole for 7 to 14 days. • All other species: Conventional dose fluconazole (150 mg).
Compromised host (e.g., poorly-controlled diabetes, immunosuppression, debilitation) and <i>Candida</i> isolate susceptible to azoles	• Oral or topical therapy for 7 to 14 days. • Pregnant patients: Lotrimin 1% cream, miconazole 2% cream, or nystatin suppositories for 7-14 days. • Avoid fluconazole in pregnancy, especially in the first trimester. • Lactating patients: Both topical azoles and oral fluconazole are compatible with breastfeeding. • Non-albicans species: <i>C. glabrata</i> is frequent vaginal colonizer, has low vaginal virulence, and rarely causes symptoms. • Preferred treatment is to use vaginal boric acid 600 mg nightly for 2-3 weeks (not in pregnant patients), or use nystatin suppositories.
Allergy to fluconazole	• Topical azole is the best treatment.

Treatment Response and Symptom Recurrence

Uncomplicated infections usually respond to treatment within a couple of days; complicated infection may require one to two weeks. There is no medical contraindication to sexual intercourse during treatment, but it may be uncomfortable until inflammation improves. Treatment of sexual partners is not indicated.

For symptom recurrence, undergo repeat evaluation to confirm and exclude other causes or mixed infection.

Management

Management depends on determining a cause, although an etiology is frequently not established. In the absence of poor hygiene, frequent washing and vaginal douching can help rather than harm. Excessive soaping of the genital area can cause a chemical vulvitis, and douching can increase the risk of vaginal and pelvic infection.

References

Sobel, J. D. (2023). *Candida albicans vaginitis* [Photograph]. UpToDate. <https://www.uptodate.com/contents/image?imageKey=PC%2F59030>

Sobel, J. D. (2023). *Candida vulvovaginitis in adults: Treatment of acute infection*. UpToDate. <https://www.uptodate.com/contents/candida-vulvovaginitis-in-adults-treatment-of-acute-infection>

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